

# AI-Powered HMS Synthetic Clinical Dataset

## Encounter Note

|                    |                    |            |            |
|--------------------|--------------------|------------|------------|
| Patient Name       | Duong Tuan Phong   | MRN        | MRN-0091   |
| DOB                | 1968-05-23         | Gender     | Female     |
| Encounter Date     | 2023-10-27         | Department | Pediatrics |
| Attending Provider | BS. Nguyen Thi Lan | Status     | active     |

Synthetic Data Warning: This document is generated for software testing, OCR parsing, chunking, embedding, access-control validation, and Graph RAG demonstrations. It is not valid medical advice and must not be used for patient care.

## Subjective

Duong Tuan Phong is a 58-year-old patient in Pediatrics presenting for follow-up of cough, wheeze, or fever with stable oxygenation. PMH includes recurrent cough or wheeze, immunizations reviewed, family educated on warning signs. Patient reports symptoms are stable overall. Denies severe chest pain, syncope, active bleeding, or new focal neurologic deficit unless otherwise documented. Current meds include albuterol inhaler 2 puffs Q4H PRN; acetaminophen 15mg/kg Q6H PRN. Allergies: latex sensitivity.

## Objective

Vitals: BP 131/81 mmHg, HR 73 bpm, RR 17/min, Temp 36.4 C, SpO2 92% RA, weight 62 kg. Exam: alert, oriented, no acute distress. Cardiopulmonary exam without unstable findings. Abdomen soft, non-tender. Extremities show no acute deformity; edema varies by department-specific condition. Labs reviewed below; medication list reconciled.

## Recent Lab Snapshot

| Date       | Analyte    | Value | Unit          | Reference Range | Status |
|------------|------------|-------|---------------|-----------------|--------|
| 2026-05-01 | Creatinine | 1.27  | mg/dL         | 0.7-1.3         | Normal |
| 2026-05-01 | eGFR       | 72.0  | mL/min/1.73m2 | >=60            | Normal |
| 2026-05-01 | Glucose    | 95.0  | mg/dL         | 70-99           | Normal |
| 2026-05-01 | HbA1c      | 6.1   | %             | <5.7            | High   |
| 2026-05-01 | BNP        | 131.0 | pg/mL         | <100            | High   |
| 2026-05-01 | AST        | 38.0  | U/L           | 10-40           | Normal |
| 2026-05-01 | ALT        | 42.0  | U/L           | 7-56            | Normal |
| 2026-05-01 | Potassium  | 4.1   | mmol/L        | 3.5-5.0         | Normal |
| 2026-05-01 | Hemoglobin | 13.5  | g/dL          | 12.0-17.5       | Normal |

## Assessment

Primary assessment: asthma or viral respiratory illness with hydration monitoring. Patient is clinically stable for ongoing management. Risk considerations include renal dosing, medication safety, fall risk, and need for timely escalation if red-flag symptoms occur.

## Plan

Monitor RR, SpO2, oral intake, and fever curve. Teach inhaler spacer technique. Return if increased work of breathing or poor intake. Provide patient education, update care team via SBAR, and document any access-relevant clinical justification before AI-assisted retrieval of PHI.

## **Detailed Care Coordination Notes**

### **Problem List**

1. asthma or viral respiratory illness with hydration monitoring. 2. Medication safety monitoring. 3. Functional status and discharge readiness. 4. Follow-up coordination with Pediatrics.

### **Safety Triggers**

Escalate to responsible clinician for SBP < 90, HR > 130, SpO2 < 92%, acute mental status change, active bleeding, severe pain, fever with neutropenia risk, or rapidly rising Cr/K.

### **AI Retrieval Tags**

access\_tags: read, summary, labs, medication, cardiology as applicable. Suggested document\_type for ingestion: encounter\_note.